

Skills

Using Personal Protective Equipment and Isolation Precautions

Checklist

Checklist

S = Satisfactory U = Unsatisfactory NP = Not Performed

Step	S	U	NP	Comments
Planning	☐	☐	☐	
Assessed the patient and reviewed the patient's history.				
Verified the health care provider's orders to determine which isolation precautions need to be used.	☐	☐	☐	
Gathered the necessary equipment and supplies.	☐	☐	☐	
Procedure Steps	☐	☐	☐	
Performed hand hygiene.				
Prepared to enter the isolation room. Choice of barrier protection will depend on the type of isolation the patient has been prescribed (e.g., contact, droplet, airborne infection) and on agency policy.	☐	☐	☐	
1. Applied a gown, made sure it covered outer garments. Pulled the sleeves down to the wrist and tied the gown securely at the neck and waist.				
2. Applied either a surgical mask, a mask with a face shield, or a fitted N95 respirator around mouth and nose. Donning an N95 respirator:				
1. Held the N95 respirator in the palm of the hand with the straps facing the floor.				
2. Placed the respirator on the face, covering the nose and mouth.				
3. Pulled the bottom strap up and over the top of the head and positioned it behind the head, below the ears.				
4. Took the upper strap and positioned it behind the head, toward the crown of the head.				
5. Molded the respirator nosepiece snugly over the bridge of the nose to obtain a tight seal.				
6. Palpated the mask to ensure an adequate seal all around the face.				
7. Followed the manufacturer's instructions for wearing the N95 respirator to ensure a snug fit.				
3. If eye wear or goggles were needed, fitted them snugly around over your face and eyes or over prescription glasses. Or used a face shield to protect the membranes of the eyes, nose, and mouth when performing tasks that generated sprays of blood or other bodily fluids.				
4. Applied clean gloves that were unpowdered. Wore latex-free gloves if allergy was present. Brought the glove cuffs over the wrists of the gown sleeves.				

Entered the patient's isolation room and closed the door. Arranged any supplies and equipment brought into the patient's isolation room. Followed agency policy for items to be reused or cleaned. If additional supplies were needed, enlisted another health care team member to hand in new supplies without entering the room.

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Introduced self to the patient and the family.

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Identified patient using two identifiers. Compared identifiers with the information on the patient's identification bracelet.

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Explained the procedure to the patient and ensured that he or she agreed to treatment. Gave patient and family an opportunity to ask questions. Assessed the patient and family for emotions related to isolation precautions.

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Administered medications as ordered while maintaining standard and isolation precautions.

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1. Provided oral medication in a wrapper or cup and then discarded the wrapper or cup in the proper trash receptacle in the patient's room.
2. Wore gloves when administering injections.
3. Discarded disposable syringes and uncapped or sheathed needles in the proper sharps receptacle in the patient's room.
4. Placed the reusable plastic syringe holder, if used, on a towel for eventual removal and disinfection after leaving the patient's room.

Provided designated care to patient while maintaining the correctly designated isolation precautions to limit the spread of contamination. Kept hands away from own face. Limited touching surfaces in the room. Removed gloves when torn or heavily contaminated, performed hand hygiene, and donned clean gloves.

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When finished caring for patient, let him or her know when return was planned.

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At the completion of care, prepared to leave the patient's isolation room. Disposed of all contaminated supplies and equipment in a way that prevented the spread of microorganisms to other people.

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Upon exiting, closed the door if Airborne Precautions were being observed or if the patient was in a negative airflow room.

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Removed PPE in order, as applicable:

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1. Inspected PPE for visible contamination, cuts, or tears before starting the doffing process. If PPE was potentially contaminated, disinfected PPE per agency policy allowed it to dry.
2. Removed gloves.
 1. If a glove is visibly soiled, cut, or torn, remove and discarded the gloves, perform hand hygiene with ABHR on bare hands, and donned clean gloves.
 2. If no visible contamination, cuts, or tears are identified on the outer surfaces of the gloves, removed and

discarded the gloves and perform hand hygiene with ABHR on bare hands.

3. If a cut or tear is detected on a glove, immediately reviewed and followed the organization's practice for occupation exposure risk.
4. Removed the first glove by grasping the cuff and pulling the glove inside out, over the hand. Held the removed glove in the remaining gloved hand. Slid the fingers of the ungloved hand under the remaining glove at the level of the wrist. Peeled the second glove off over the first glove. Then discarded both gloves in the proper container without touching the outer surface of the gloves, as it is considered contaminated.
3. Removed eye wear, face shield, or goggles from the back of the head by lifting the headband or earpieces up and over the head. Avoided touching the outer surface of the eye protection or face shield, if worn. Discarded the item in the proper container. Disinfect reusable eye protection after each use per the organization's practice.
4. Removed the gown by first untying the neck strings or VELCRO® closure and then untying the back strings. Allowed the gown to fall from the shoulders; touched the inside of the gown only. Removed hands from gown sleeves without touching the outside of the gown; it is considered contaminated. Held the gown inside at the shoulder seams and folded the gown inside out into a bundle; discarded it in the linen receptacle.
5. Removed mask. If the patient is in a negative-pressure AIIR, left the room and closed the door before the N95 respirator or PAPR (as applicable) was removed for a patient in a negative-pressure AIIR. If the mask was secured over the ears, removed the elastic from ears and pulled the mask away from the face. For a tie-on mask, untied the bottom string and then the top strings before pulling the mask away from the face. Dropped the used mask into the trash receptacle, being careful not to touch the outer surface of the mask, or stored per the organization's practice. Note: for patients who were under airborne precautions, the N95 mask or respirator were removed outside of the patient's room. Donned clean gloves, tilted head slightly forward, and removed without touching the front of the mask or respirator. Placed N95 mask or respirator in appropriate container for reuse or disposal per the organization's practice. Removing the N95 respirator:
 1. Tilted the head forward.
 2. Using both hands, grabbed the bottom strap, pulling it to the sides and over the head.
 3. Grabbed the upper strap with both hands and pulled it to the sides and over the head. Kept tension on the upper strap during removal to let the mask fall forward.
 4. Discarded the N95 in the proper trash receptacle or store per the organization's practice. Discarded the respirator if it became wet or damaged.
 5. Removed and disposed of gloves.
6. Performed hand hygiene.

Documented any procedures performed, how patient was handling the isolation, and any education offered or reinforced.



Learner: _____
Evaluator: _____
Date: _____

Signature: _____
Signature: _____